

NeuroPathway & MediSense

Clinical Safety, DTAC & NHS Assurance Framework Prompt

DCB0129 | DCB0160 | DTAC | AI Governance | UK GDPR | DSPT | NHS Cyber Assurance

You are acting as a Senior NHS Clinical Safety Officer, DTAC Assessor, NHS Digital Assurance Lead, AI Governance Specialist, Data Protection Officer, and Health IT Risk Management Consultant.

Your task is to create a complete NHS-ready Clinical Safety and Assurance Evidence Pack for the NeuroPathway and MediSense platforms.

The evidence pack must align with:

- DCB0129: Clinical Risk Management for Manufacturers of Health IT Systems
- DCB0160: Clinical Risk Management for Deployment and Use of Health IT Systems
- NHS DTAC: Digital Technology Assessment Criteria
- NHS AI Assurance Principles
- UK GDPR
- Data Protection Act 2018
- NHS Data Security and Protection Toolkit
- NHS Cyber Assessment Framework principles
- NHS Information Governance requirements
- Clinical safety, safeguarding, accessibility, equality, and human oversight expectations

Platform Overview

NeuroPathway

NeuroPathway is an AI-supported early identification and evidence platform designed to help parents, schools, health professionals, and social care professionals identify repeated patterns in behaviour over time.

The platform helps generate structured, EHCP-ready evidence by collecting everyday observations across education, home, health, and social care settings.

NeuroPathway does not diagnose.

NeuroPathway does not replace professional judgement.

NeuroPathway supports earlier identification, clearer evidence, multi-agency working, and needs-led support.

MediSense

MediSense is an NHS-facing AI-assisted care summary layer that supports safer handovers and admissions.

It provides clinicians with a concise summary of key care needs, including:

- Medication information
- Dose and reason for medication
- Diagnoses
- Known risks
- Communication preferences
- Reasonable adjustments
- Calming strategies
- Care needs
- Safeguarding alerts where appropriate
- Relevant patient history

MediSense does not make clinical decisions.

MediSense does not prescribe, diagnose, triage, or override clinicians.

MediSense acts only as a summarisation and decision-support tool.

Human professionals remain responsible for all clinical decisions.

Core Safety Position

The AI must be treated as assistive technology only.

The platform must clearly state:

- AI outputs are not clinical decisions.
- AI outputs must be reviewed by a qualified professional.
- AI must not be used as the sole basis for diagnosis, safeguarding decisions, medication decisions, EHCP decisions, discharge decisions, or treatment decisions.
- All outputs must show source evidence where possible.
- Where information is missing, uncertain, conflicting, or outdated, the system must clearly say so.
- The system must refuse to invent information.
- The system must maintain audit trails, version history, data provenance, and user accountability.

Required Deliverables

Create a full NHS-style assurance pack with the following sections.

1. Clinical Risk Management Plan

Create a formal Clinical Risk Management Plan covering:

- Purpose and scope
- Intended use
- Excluded use
- Clinical safety governance structure
- Named Clinical Safety Officer responsibilities
- Product owner responsibilities
- Deployment organisation responsibilities

- Risk assessment process
- Hazard identification process
- Hazard review process
- Safety incident management
- Clinical safety escalation routes
- Change control process
- AI model change control
- User training requirements
- Clinical safety documentation control
- Post-deployment monitoring
- Periodic safety review schedule
- Go-live safety approval process
- Safety sign-off process

Include a clear distinction between:

- DCB0129 responsibilities for NeuroPathway / MediSense as the manufacturer
- DCB0160 responsibilities for the NHS organisation or partner deploying the technology

2. Clinical Hazard Log

Create a detailed clinical hazard log with at least 40 hazards.

Use this table structure:

- Hazard ID
- Hazard Description
- Platform Affected: NeuroPathway, MediSense, or Both
- Cause
- Potential Clinical / Care Impact
- Severity
- Likelihood
- Initial Risk Rating
- Existing Controls
- Additional Mitigations Required
- Residual Risk Rating
- Safety Owner
- Review Frequency
- Evidence Required

Include hazards covering:

- AI hallucination
- Incorrect summary generation
- Missing medication information
- Outdated medication information
- Incorrect dose information
- Incorrect patient matching
- Missing diagnosis

- Misinterpreted behaviour patterns
- EHCP evidence misinterpretation
- Over-reliance on AI output
- Professional misunderstanding
- Parent misunderstanding
- Safeguarding concern not escalated
- False safeguarding concern generated
- Incorrect RAG rating
- Alert fatigue
- Accessibility barriers
- Bias in AI outputs
- Poor data quality
- Incomplete data integration
- Interoperability failure
- FHIR/API failure
- System downtime
- Cyber incident
- Unauthorised access
- Incorrect user role access
- Lack of audit trail
- Model drift
- Uncontrolled AI model update
- Incorrect confidence score
- Failure to show data source
- User bypassing professional review
- Inadequate staff training
- Poor handover use
- Patient distress caused by incorrect summary
- Delayed care due to missing information
- Inappropriate exclusion or SEND decision
- Over-medicalisation of behaviour
- Failure to record reasonable adjustments
- Failure to identify escalating crisis
- Misuse outside intended purpose

3. Clinical Safety Case Report

Create a formal NHS-style Clinical Safety Case Report including:

- Executive Summary
- Product Description
- Intended Use
- Excluded Use
- Clinical Context
- User Groups
- Patient / Child / Service User Groups
- Clinical Safety Objectives
- Safety Claims
- Evidence Supporting Each Safety Claim

- Hazard Analysis Summary
- Residual Risk Summary
- Benefits vs Risks
- Known Limitations
- Assumptions and Dependencies
- Deployment Conditions
- Required User Training
- Required Local Configuration
- Safety Incident Reporting
- Post-Deployment Monitoring
- Clinical Safety Conclusion
- Clinical Safety Officer Sign-Off Section
- Manufacturer Sign-Off Section
- Deploying Organisation Sign-Off Section

The safety case must make clear that the systems are designed to support, not replace, professional judgement.

4. DTAC Clinical Safety Evidence Pack

Create a DTAC evidence mapping table showing:

- DTAC requirement
- Evidence required
- Current status: Red / Amber / Green
- Evidence NeuroPathway / MediSense can provide
- Gaps to close
- Recommended action
- Owner
- Priority

Include evidence for:

- Clinical safety
- Data protection
- Technical security
- Interoperability
- Usability and accessibility
- Equality impact
- Clinical effectiveness
- AI assurance
- Risk management
- Information governance
- Cyber security
- Service support model
- Business continuity
- Incident response
- User training
- Supplier responsibilities

- Deployment responsibilities
-

5. DCB0129 Compliance Checklist

Create a manufacturer-focused DCB0129 checklist for NeuroPathway and MediSense.

Include:

- Clinical Safety Officer appointed
- Clinical Risk Management System documented
- Clinical Risk Management Plan completed
- Hazard Log created
- Clinical Safety Case Report completed
- Safety incident process created
- Change control process documented
- Clinical safety file maintained
- Manufacturer safety responsibilities defined
- User training materials produced
- Implementation guidance created
- Release notes and version control in place
- AI model update process documented
- Post-market surveillance process in place
- Residual risks communicated to deploying organisations

For each item include:

- Status
 - Evidence needed
 - Owner
 - Priority
 - Notes
-

6. DCB0160 Deployment Checklist

Create a deployment-focused DCB0160 checklist for NHS organisations, schools, local authorities, ICBs, or partner organisations using the platform.

Include:

- Local Clinical Safety Officer appointed
- Local deployment risk assessment completed
- Local workflow impact assessed
- Staff training completed
- Local hazard log reviewed
- Manufacturer safety case reviewed
- Local clinical safety case completed
- Information governance approval completed

- DPIA completed
- Cyber assurance reviewed
- User access controls configured
- Incident reporting route confirmed
- Business continuity plan confirmed
- Go-live approval completed
- Post-go-live monitoring scheduled

For each item include:

- Responsible organisation
- Evidence needed
- Status
- Priority
- Notes

7. AI Safety and Assurance Controls

Create a dedicated AI Safety Control Framework covering:

- Human oversight
- Explainability
- Transparency
- Confidence scoring
- Data provenance
- Source traceability
- Safe refusal behaviour
- Hallucination prevention
- Bias monitoring
- Equality impact
- Model drift monitoring
- AI output audit logs
- Prompt governance
- Model version control
- AI safety testing
- Red-team testing
- Clinical review of AI outputs
- User warnings
- Professional accountability
- Escalation where risk is detected

For each AI control include:

- Risk addressed
 - Control description
 - Evidence required
 - Monitoring method
 - Owner
-

8. Safeguarding and SEND Safety Controls

For NeuroPathway, create a specific safeguarding and SEND safety section covering:

- The platform does not diagnose.
- The platform does not replace SEND professionals.
- The platform supports evidence gathering only.
- EHCP-ready evidence must be reviewed by humans.
- Safeguarding alerts must be routed to the correct professional.
- Behaviour must not be labelled negatively.
- Outputs must use strengths-based, needs-led language.
- The system must avoid blaming the child, parent, or school.
- The system must identify what support has already been tried.
- The system must identify what worked and what did not.
- The system must avoid making unsupported recommendations.

Create a risk table for SEND-specific risks.

9. Medication and Admission Safety Controls

For MediSense, create a specific clinical safety section covering:

- Medication accuracy
- Medication source verification
- Dose verification
- Allergy information
- Diagnosis information
- Reasonable adjustments
- Communication needs
- Known calming strategies
- Crisis prevention plans
- Admission summary safety
- Handover safety
- Discharge or sign-out kill switch
- No long-term unnecessary AI retention
- Audit trail of who viewed the summary
- Escalation if medication information is missing or conflicting

Create a medication safety hazard table.

10. Governance Framework

Create an NHS-style governance framework including:

- Clinical Safety Committee
- Information Governance Lead

- Data Protection Officer
- Clinical Safety Officer
- Safeguarding Lead
- AI Governance Lead
- Cyber Security Lead
- Product Owner
- Patient / Parent / Public Involvement representative
- NHS deployment partner representative

Define:

- Meeting frequency
- Decision-making authority
- Escalation routes
- Reporting structure
- Risk acceptance process
- Safety sign-off process
- Incident review process
- Continuous improvement process

11. NHS Pilot Readiness RAG Assessment

Create a Red / Amber / Green pilot readiness assessment for:

- Clinical Safety
- DCB0129 readiness
- DCB0160 readiness
- DTAC readiness
- Data Protection
- DPIA readiness
- Cyber Security
- DSPT readiness
- AI Assurance
- Safeguarding
- Accessibility
- Interoperability
- User Training
- Procurement Readiness
- NHS Partner Readiness
- Evidence of Benefit
- Evaluation Plan

For each category include:

- Current rating
- Reason for rating
- Evidence already available
- Gaps
- Recommended next action

- Priority
-

12. Required Evidence Register

Create a documentation register listing every document needed before an NHS pilot.

Include:

- Document name
- Purpose
- Standard supported
- Owner
- Status
- Priority
- Review date

Documents should include:

- Clinical Risk Management Plan
 - Hazard Log
 - Clinical Safety Case Report
 - DPIA
 - Data Processing Agreement
 - Privacy Notice
 - Cyber Security Policy
 - Access Control Policy
 - Incident Response Plan
 - Business Continuity Plan
 - AI Governance Policy
 - Safeguarding Policy
 - Equality Impact Assessment
 - Accessibility Statement
 - User Training Guide
 - Implementation Guide
 - Pilot Evaluation Framework
 - Post-Deployment Monitoring Plan
 - Change Control Policy
 - Model Monitoring Policy
-

13. Final NHS-Ready Summary

End with a professional executive summary suitable for ICBs, NHS Trusts, local authorities, health innovation networks, procurement teams, and clinical safety reviewers.

The summary must explain that NeuroPathway and MediSense have been designed with safety, governance, transparency, and human oversight at the centre.

Use professional but human language.

The tone should be credible, clear, NHS-aligned, and suitable for a founder-led innovation seeking pilot support.

Do not overclaim compliance unless evidence is complete.

Use wording such as:

"Designed to align with" "Prepared for clinical safety review" "Evidence pack in development" "Requires CSO review and sign-off" "Subject to local DCB0160 deployment assessment" "Human oversight remains mandatory" "AI supports but does not replace professional judgement"

Avoid saying:

"Fully compliant" "NHS approved" "Clinically validated" "Guaranteed safe" "Diagnostic tool" "Automated decision-maker"

The final output should be structured, professional, and suitable to become the foundation for a formal NHS Clinical Safety File.